Reviewer 2 v.1

Comments to the Author

Dear Editor,

thank you for the opportunity to review this manuscrit by Qianqian Chen et al. entitled “Increased Risk of Lung Cancer in Combined Pulmonary Fibrosis and Emphysema Compared with Idiopathic Pulmonary Fibrosis: A Systematic Review and Meta-Analysis”. This manuscript is a systematic review and meta-analysis focused on the prevalence of lung cancer in patients with CPFE compared to patients with IPF. The authors found that a significantly higher prevalence of lung cancer in patients with CPFE than those with IPF alone.

In the method section the inclusion criteria mentioned as “ Outcome: risk of developing lung cancer in CPFE” need to be better clarified. It is not clear what the authors wanted to include with this

The manuscript need to be revised by a native English speaker since it appears not clearly written and with several misunderstanding or contradictory concepts. In particular much attention must be paid in the method and results section.

What did the author want to assert (in the discussion) with “… we only included CPFE patients with pulmonary fibrosis of IPF subtype in our meta-analysis and performed a subgroup analysis for different emphysema thresholds” ? It doesn’t sound clear enough and it appears that method are not properly conducted. Indeed they have never mentioned in the selection criteria that they inclued studies according to the presence of a specific radiologic appearance. Did the authors excluded “probable UIP pattern “ or alternative o indeterminate pattern at HRCT? If yes why?

Since diagnostic criteria for defining CPFE is not fully standardized there is a great heterogeneity of patients included in such studies selected for the meta-analysis that may contribute to influence the interpretation of data. In particular the lack of details regarding the extension of emphysema is a great limitation of this attempt to select the studies. Indeed the risk to overestimate the number of patients with CPFE instead of only IPF with some HRCT feature of centrilobular emphysema is very high.